

ST. JUDE MEDICAL CENTER - DISCHARGE SUMMARY

PATIENT DISCHARGE SUMMARY

St. Jude Regional Medical Center
Department of Internal Medicine

PATIENT DEMOGRAPHICS

Patient Name: John Doe
Age: 67 | Gender: Male
MRN: 984210-A
Admission Date: October 14, 2024
Discharge Date: October 18, 2024

CHIEF COMPLAINT & ADMISSION REASON

Shortness of breath, productive cough, and high-grade fever (102.4 F) for 3 days prior to admission.

PAST MEDICAL HISTORY

- Essential Hypertension (10 years, managed on Lisinopril)
- Hyperlipidemia

ALLERGIES

Penicillin (Severe rash and hives documented in 2018)

DIAGNOSIS & DISPOSITION

Primary Diagnosis: Community Acquired Pneumonia (Right Lower Lobe)
Secondary Diagnosis: Essential Hypertension
Disposition: Discharged home in stable condition with outpatient follow-up.

LABORATORY & DIAGNOSTIC FINDINGS

- WBC Count: 15.2 K/uL (Reference Range: 4.0 - 11.0 K/uL) [HIGH]
- Hemoglobin: 13.4 g/dL (Reference Range: 12.0 - 16.0 g/dL) [NORMAL]
- C-Reactive Protein (CRP): 82.0 mg/L (Reference Range: 0.0 - 5.0 mg/L) [HIGH]
- Chest X-Ray: Right lower lobe alveolar consolidation consistent with bacterial pneumonia.

DISCHARGE MEDICATIONS

- Amoxicillin 500 mg PO three times daily for 7 days (Note: Verify allergy status before dispensing)
- Lisinopril 10 mg PO once daily
- Acetaminophen 500 mg PO every 6 hours as needed for fever/pain

RECOMMENDED NEXT STEPS & DISCHARGE INSTRUCTIONS

- Complete full 7-day course of antibiotic therapy.
- Follow up with Primary Care Physician in 3 to 5 days.
- Return to Emergency Department immediately if chest pain, severe dyspnea, or persistent fever (>101 F) occurs.